A Guide for Using AI Scribes in the Medical Sector

AWO & Digital Rights Watch

Introduction

This guidance is for general practitioners (GPs) and medical practices using ‘AI scribes’ (sometimes referred to as ‘ambient AI’ or ‘AI notetakers’) for their work, in particular to support note taking and document drafting.

As generative AI continues to develop and its uptake increases, organisations across industries are experimenting with different use cases for the technology. In the medical field specifically, there has been a growing use of AI scribes. These AI systems can be used to transcribe conversations and generate notes. They record conversations between patients and doctors, and dictation by doctors, and generate clinical or other types of notes in standard or tailored formats for doctors to review and export to their practice’s electronic health record (EHR) system or document management software.

While GPs ultimately remain responsible and liable for the use of these tools, it is important that all relevant staff are provided with appropriate guidance and training on selecting and using AI scribes. This guide therefore seeks to provide practical information to GPs and medical practices in Australia which are considering, or already using, AI scribes. This guide includes:

- An accessible explanation of what AI scribes are and how they work.
- The risks that AI scribes pose and the potential resulting harms for patients.
- Practical steps to ensure the responsible use of AI scribes by GPs and medical practices.
- FAQs for GPs and medical practices.
- Vendor due diligence checklist to help you select an appropriate AI scribe tool from a data protection perspective.
- Templates that can be tailored to your circumstances, including a patient consent script for use of an AI scribe, a waiting room notice on AI scribes, and suggested wording for your privacy policy when using an AI scribe.

This guide is intended for general information purposes only and does not constitute legal advice. It should not be relied upon as a substitute for professional legal advice.

Quick Start Guide

Since 2025, there has been significant uptake in the use of medical AI scribes by GPs in Australia. Use of these AI systems comes with both potential benefits and risks. While AI could provide productivity gains and improve how individuals and teams work, it can also introduce new threats and harms originating from the way it is used and the nature of the technology itself. Recognition of this reality is especially important for highly regulated practices such as healthcare, which involves the handling of sensitive and personal information.

The use of AI scribes presents significant privacy and consent challenges under Australian federal and State laws, particularly where sensitive health information is shared with third-party vendors, processed offshore, or collected without meaningful, informed and voluntary patient consent. Concerns have also been raised about how the use of these tools may impact what patients reveal in consultations (e.g., self-censorship due to being 'recorded'), and how GPs attend to the patients they are seeing.[1]

These systems have also been known to incorrectly transcribe (or completely omit) key patient information, and exhibit biases against certain groups, including minority women.[2] Accuracy and bias risks can directly impact patient care, leading to incorrect diagnoses, unequal outcomes for diverse populations, and errors in clinical records if outputs are not rigorously reviewed.

Given the sensitivity of the data being collected from patients, and the importance of accurate diagnoses and treatment, medical professionals must ensure they use digital tools in an ethical and diligent manner. Doctors remain responsible and liable for the tools they use, making it essential to implement strong governance controls, including vendor due diligence, clear consent processes, training, and ongoing monitoring, to ensure safe and compliant use.

Table 1: Quick Guide Action Plan*

Step	***What***	***How***
Step 1	Implement a selection policy	Implement a policy or standard procedure for selecting AI tools/software. If already in place, it should be reviewed and kept up-to-date.
Step 2	Select a tool and vet the vendor	Define the reason for introducing the tool and ensure it is justified. Consider the limitations of both the tool and its usage. After selecting the AI tool/software, undertake vendor due diligence to ensure appropriate safeguards are in place. See Annex IV for a due diligence checklist. You must tailor this as appropriate.
Step 3	Train staff	Undertake staff training on how to use the tool, how it should be configured etc. Keep a record of training.
Step 4	Update policies and notices	Prior to implementing the tool, update your privacy policy and notices about AI use in

[1] Elizabeth Deveny, "AI scribes in general practice: support, silence and the shape of care", InSight+ (MJA) (21 July 2025) https://insightplus.mja.com.au/2025/28/ai-scribes-in-general-practice-support-silence-and-the-shape-of-care accessed 12 May 2026.

[2] See, for example, T Draper and others, "Clinical AI Scribes in Primary Care: Accuracy, Error Severity and Implications for Clinical Practice" (2025) 1(1) *BMJ Digital Health & AI* e000092. https://bmjdigitalhealth.bmj.com/content/1/1/e000092; Joshua Biro and others, "Accuracy and Safety of AI-Enabled Scribe Technology: Instrument Validation Study" (2025) 27 *J Med Internet Res* e64993 https://doi.org/10.2196/64993; Erin Palm and others, "Assessing the Quality of AI-Generated Clinical Notes: Validated Evaluation of a Large Language Model Ambient Scribe" (2025) 8 *Front Artif Intell* 1691499 https://doi.org/10.3389/frai.2025.1691499 (all accessed 12 May 2026).

		compliance with the Australian Privacy Principles (APPs) and relevant Office of the Australian Information Commissioner (OAIC) guidance. Put up signs in your medical practice about the use of AI. See Annex II and III for some template guidance wording. You must tailor this as appropriate.
Step 5	Obtain and document consent	Obtain consent prior to every consultation. Document the consent provided and always give patients the right to refuse. See Annex I for suggested consent wording. You must tailor this as appropriate.
Step 6	Review notes for accuracy	GPs must always review AI-generated notes for accuracy, including omissions.
Step 7	Monitor on an ongoing basis	Engage in continual and ongoing monitoring and assessment of AI scribe use, including for accuracy and patient feedback.

*This is a high-level plan and does not replace the detailed guidance contained in the main text.

Table 2: Risks and mitigations for GPs and medical practices

Risk	***Risk Category***	***Potential Harm to Patients***	***What You Can Do***
Inaccurate transcripts of patient and/or GPs' speech (accents, dialects, medical jargon).	Accuracy	Wrong diagnoses, inappropriate treatments, or missed conditions due to incorrect information in patients' medical records.	Always review the full AI-generated transcript against your own recollection of the consultation. Pay special attention to medication names, dosages, and clinical terms. Flag recurring errors to your vendor.
AI hallucinations: fabricated clinical details never discussed in the consultation.	Accuracy	False symptoms or conditions entered into the patient record, leading to harmful clinical decisions (e.g., incorrect mental health flags)	Compare every AI-generated summary against the original audio or your notes before saving it to the patient record. Treat the AI output as a draft, never as the final record.
Racial and gender bias in transcription and	Bias	Systematically poorer-quality recordings of patients from minority backgrounds, women, or	Monitor whether the tool performs differently for patients with diverse backgrounds or accents.

summarisation outputs.		those with disabilities, leading to unequal care and entrenched health disparities. A recent study funded by the UK's National Institute for Health and Care Research found that when social workers used Google's AI model 'Gemma', the case note summaries it generated downplayed women's physical and mental health issues as compared to men's.[3]	Report patterns of bias to your practice manager and vendor. Supplement AI-generated notes with your own observations for patients where you notice lower accuracy.
Over-reliance on AI-generated notes without adequate review.	Oversight	Errors, omissions, or biased language go undetected in the medical record, compounding over time and affecting future clinical decisions.	Allocate dedicated time after each consultation to thoroughly review the AI-generated record. Do not copy-paste outputs directly into the patient management system without reading them in full.
Lack of genuine informed consent from patients before recording.	Privacy	Patients being unaware their consultations are being recorded and processed by AI, undermining trust in the doctor-patient relationship and violating privacy expectations. Even where consent is provided, it can be argued that due to the power imbalance between doctors and patients, consent cannot be truly genuine.	Explain to patients in plain language what the AI scribe does, how their data is used, and who can access it. Obtain explicit consent before activating recording. Offer a genuine alternative (manual notes) for patients who decline. Emphasise the ability to say 'no' or to request at any time that the recording be turned off, including when a patient may want to discuss something sensitive.
Covert or insufficiently disclosed collection of patient data.	Privacy	Breach of privacy obligations under the Australian Privacy Act (APP3 and APP5); breach of State surveillance devices laws, leading to the erosion of patient trust and potential regulatory action against the practice.	Ensure your practice has a clear privacy notice that specifically covers AI transcription. Display signage in the waiting room. Include AI scribe use in your collection notification to patients under APP5.

[3] Sam Rickman, 'Evaluating gender bias in large language models in long-term care', BMC Medical Informatics and Decision Making Vol. 25 Article 275 (11 August 2025) https://link.springer.com/article/10.1186/s12911-025-03118-0.

Sensitive health data stored or processed offshore by third-party sub-processors.	Privacy	Patient health data (including on mental health, disability, and reproductive information) is exposed to foreign jurisdictions with weaker privacy protections, increasing the risk of breaches or misuse.	Before procuring an AI scribe, confirm where the data is stored and processed. Require Australian-based data residency or equivalent protections in your vendor contract. Conduct a privacy impact assessment.
Loss of professional clinical judgement and deskilling over time.	Autonomy	Doctors become less skilled at clinical note-taking, observation, and analytical reasoning over time, reducing care quality when AI tools are unavailable or fail.	Continue to exercise your own clinical judgement when reviewing AI outputs. Periodically write consultation notes manually to maintain your documentation skills. Use AI as an assistant, not as a replacement for your professional analysis.
No right to deletion of AI-processed patient data in Australia.	Governance	Patients are unable to have inaccurate or unwanted AI-generated records corrected or removed, leading to persistent errors in their medical history that affect their future care and insurance outcomes.	Establish a clear process for patients to request the correction or removal of AI-generated records. Understand your vendor's data retention and deletion policies. Ensure your practice's data governance policy covers AI-generated content.
No regulation of AI transcription tools as medical devices.	Governance	AI tools providing diagnostic-adjacent services without meeting medical device standards, exposing patients to unregulated clinical risk.	Check whether your AI scribe vendor has any clinical safety certifications or compliance standards. Advocate through your professional body for clearer regulation of AI tools in Australian healthcare. Do not use AI scribes for diagnostic functions they are not designed for.
Excessive data collection beyond clinical necessity (ambient recording).	Privacy	Sensitive personal information (overheard conversations, incidental disclosures) captured by ambient recording, potentially used for secondary purposes such as insurance profiling or commercial analytics.	Only activate the AI scribe during the clinical consultation, not before or after. Use the tool in a private room to avoid capturing third-party conversations. Review your vendor's data minimisation practices and ensure data is not used for secondary purposes.

Non-person-centred or overly clinical language in AI-generated records.	Autonomy	Patient records that use dehumanising, overly academic, or impersonal language, making them inaccessible to patients exercising their right to access their own records, and undermining patient-centred care.	Edit AI-generated notes to use patient-friendly, person-centred language before saving. Use prompts or templates that instruct the AI to write in plain language. Remember that patients have a right to access their records and should be able to understand them.
Self-censorship by patients – a ‘chilling effect’ on speech.	Autonomy	Patients may self-censor and choose not to disclose certain information when they are being recorded – whether about domestic violence, child protection, sexual health, stigmatised conditions, drug and alcohol consumption, or other sensitive issues. Experts have noted that healthcare is deeply human; consultations are more than mere transactions or information exchange.[4] The introduction of AI into consultations changes not only what gets recorded, but what gets said.[5]	Really slow down to obtain consent and explain why you are using the tool. Provide the necessary information and remain available to answer any questions a patient may have. Emphasise the patient’s ability to say ‘no’, or to request at any time that the recording be turned off, including where they may want to discuss something sensitive.

1 A Primer on AI Scribes

1.1 What are AI scribes?

AI scribes are software systems designed to use recordings of conversations or dictation to generate written notes, summaries, communications, or structured records, either in real-time or shortly after the recording ends. These systems make use of audio inputs either directly from hardware devices or from existing recordings to produce the written outputs. They are audio-in, text-out systems.

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[4] Elizabeth Deveny, “AI scribes in general practice: support, silence and the shape of care”, InSight+ (MJA) (21 July 2025) https://insightplus.mja.com.au/2025/28/ai-scribes-in-general-practice-support-silence-and-the-shape-of-care accessed 13 May 2026.

[5] Elizabeth Deveny, “AI scribes in general practice: support, silence and the shape of care”, InSight+ (MJA) (21 July 2025) https://insightplus.mja.com.au/2025/28/ai-scribes-in-general-practice-support-silence-and-the-shape-of-care accessed 13 May 2026.

The features that AI scribe providers offer varies. Generally, AI scribes perform two main functions:

- **Transcription.** AI scribes convert the audio captured from a conversation/dictation or an existing recording into a transcript, often automatically identifying the individual speakers involved and the language used.
- **Document drafting.** AI scribes can then produce a summary of the conversation/dictation, including identification of the different topics discussed, the decisions made and any action items set, as well as structuring text in different formats (e.g., clinical consultation notes, care plans, discharge or referral letters, coding documentation, treatment plans) according to default or clinician-generated templates.

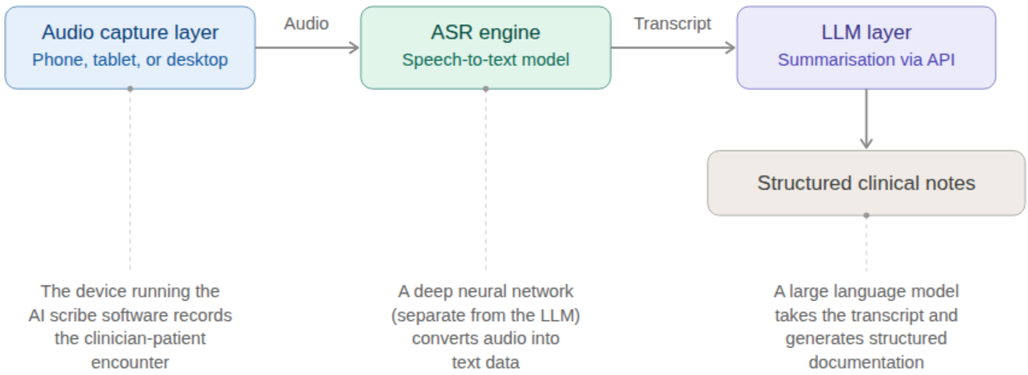
Once notes and documents have been generated by the AI scribe, they can be reviewed and transferred by the clinician to the EHR system used by their hospital, or other document management software they usually rely on.

In addition to these ‘core’ functions, AI scribes can also provide additional functionalities, such as personalisation, longitudinal documentation generation, and chatbots.

AI scribes combine automatic speech recognition (ASR) – which converts audio recordings into written transcripts – and generative AI (a large language model, or LLM) to process transcripts and produce notes or other clinical documents. They are trained on vast quantities of data and generate outputs by calculating the most statistically probable next word, given what comes before.

This aspect is critical to understanding the nature of the risks. As neither system understands language in any meaningful sense, they operate by making ‘calculated guesses’, at scale and at speed, about what word is most likely to follow the one prior.

The probabilistic nature of these systems is the single most important thing for GPs and medical practices to understand. Because they generate text based on statistical likelihood rather than factual recall, they are prone to make mistakes based on the training data that was used to build them, and/or as a consequence of the probabilistic nature of their functioning. This can lead to plausible-sounding content being produced that didn’t come up in the conversation. This is commonly referred to as a ‘hallucination’. Conversely, it can also lead to transcripts having critical information removed because it was deemed ‘background noise’.



1.2 How are AI scribes used in the medical context?

In the medical context, AI scribes are typically used as documentation assistants, so that medical professionals do not have to document the consultation from memory afterwards. This can help medical professionals maintain detailed notes of their conversations with patients. We are likely to see these tools evolve to include diagnostic capabilities, schedule follow-ups with patients, and generate care plans.

The two scenarios below set out how AI scribes are generally being used by medical professionals to help with note-taking and customary administrative tasks.

Scenario 1: A GP consultation in a busy suburban clinic
A GP is working in a busy suburban practice in western Sydney. They see around 30 patients a day across a mix of acute presentations, chronic disease management, and mental health consultations. Using an AI scribe app on their tablet, the GP records their 15-minute conversation with a patient. The tool listens as the GP and patient cover blood glucose levels, medication side effects, dietary changes, and a referral to a podiatrist. Following the appointment, the app generates a structured draft consultation note. The GP reviews the draft for accuracy and completeness and then saves it to the practice's document management system.

Scenario 2: A specialist dermatology appointment with a complex case
A dermatologist in a private clinic in Melbourne is seeing a patient for a follow-up after a biopsy the previous week. The consultation involves reviewing the pathology results, providing a diagnosis, discussing surgical excision options, explaining margins and staging, and addressing the patient's anxiety about the diagnosis. The dermatologist activates their AI scribe at the start of the appointment. The tool captures the entire conversation, including the patient's questions about prognosis, the discussion of a wide local excision, and the plan to treat with an associated referral. After the patient leaves, the AI scribe generates a comprehensive consultation note organised under standard headings: history, examination findings, results reviewed, diagnosis, management plan, and patient education provided.

2 The Potential Benefits of AI Scribes in Healthcare

While further research is required, our interviews with GPs in Australia suggest a range of benefits regarding the use of these tools. Our interviews show there is some agreement among GPs that AI scribes can benefit both patients and GPs.[6] GPs say that AI scribes allow them to focus more on their patients, improve patient-doctor relationships, reduce their admin burden, increase their capacity to see patients, and enable a better work-life balance.[7]

Some research, however, moderates at least some of these claims by suggesting productivity gains may be more modest than they first appear.[8] Concerns have also been raised about the impact on the patient experience including doctors remembering less about their patients' histories, transcription errors, errors in accent recognition, consultations remaining just as rushed,[9] and the amount of time spent by GPs reviewing AI-generated notes.

Capturing ongoing evidence will be essential to understanding the long-term and system-wide impacts of AI transcription tools in clinical settings. Achieving this will require coordinated efforts by researchers, civil society, and policymakers.[10]

3 The Risks of AI Scribes in Healthcare

Please see the table above, in the Quick Start Guide, which provides an overview of risks and mitigations. Below, we elaborate on the dominant categories of risk.

[6] Essential Research and Digital Rights Watch, AI Transcription in Healthcare Workshop Insights Report, 29 April 2026.
[7] Essential Research and Digital Rights Watch, AI Transcription in Healthcare Workshop Insights Report, 29 April 2026.
[8] A US study, for example, tracked AI scribe use across five hospitals for more than two years and found that this led to only a 16-minute reduction in time spent on documentation (around -10%), leading to just 0.5 additional patient visits per week. Lisa Rotenstein and others, "Changes in Clinician Time Expenditure and Visit Quantity With Adoption of Artificial Intelligence-Powered Scribes: A Multisite Study" (2026) JAMA https://doi.org/10.1001/jama.2026.2253, accessed 13 May 2026. Other studies show similar figures for documentation and EHR use, and documentation time reduction. See, for example, Jye Tan and others, "Impact of an Ambient AI Scribe Among Clinicians and Patients: Real-World Prospective Observational Time-Motion Study" (2026) 14 JMIR Medical Informatics e85580 https://medinform.jmir.org/2026/1/e85580, accessed 13 May 2026; Cheryl D. Stults and others, "Evaluation of an Ambient Artificial Intelligence Documentation Platform for Clinicians" (2025) 8(5) JAMA Network Open e258614 https://doi.org/10.1001/jamanetworkopen.2025.8614, accessed 13 May 2026.
[9] Elizabeth Deveny, "AI scribes in general practice: support, silence and the shape of care", InSight+ (MJA) (21 July 2025) https://insightplus.mja.com.au/2025/28/ai-scribes-in-general-practice-support-silence-and-the-shape-of-care, accessed 12 May 2026.
[10] Lara Groves and Oliver Bruff, *Scribe and Prejudice? Exploring the Use of AI Transcription Tools in Social Care* (Ada Lovelace Institute, 11 February 2026) https://www.adalovelaceinstitute.org/wp-content/uploads/pdfs/33783/scribe-and-prejudice.pdf?v=1778241451, accessed 12 May 2026, p 10.

3.1 Accuracy and reliability

Inaccurate transcriptions or summaries/notes can have significant downstream consequences for patients regarding their diagnosis or the treatment they receive. Research shows that AI scribes generally make more mistakes than doctors, not only in terms of transcription or note errors, but also omissions or the inclusion of text that was never spoken or does not follow directly from the consultation notes.[11]

Some AI scribe providers suggest that their systems are capable of autonomously 'making choices' about what to include in transcriptions, and have the ability to identify and remove small talk and non-pertinent information, retaining only clinically relevant details. However, these kinds of AI systems can make mistakes and errors. For instance, AI scribes may struggle with contextual understanding – misinterpreting a patient's hypothetical statement as a confirmed diagnosis,[12] or confusing historical and current symptoms. This can lead to critical documentation errors that might eventually extend to other parts of the patient's health record, or to the clinical information systems of other clinicians.[13]

Additionally, AI scribes may exhibit variable performance across speakers with different accents, regional dialects, English as a second language, and speech disorders or impairments.[14]

Inaccuracy risks also extend to AI scribes that provide diagnostic support functionalities, ranging from medical coding suggestions to inferring treatment options based on the content of the consultation. Inaccurate transcriptions can easily lead to incorrect or inappropriate diagnostic suggestions or determinations from AI scribes. It is therefore imperative that doctors using these tools diligently review and check the generated content and recommendations before relying on them. Doctors should recall they are ultimately responsible for the documentation they produce and the diagnoses they give to their patients.

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[11] See, for example, Thomas C. Draper and others, "Clinical AI Scribes in Primary Care: Accuracy, Error Severity and Implications for Clinical Practice" (2025) 1(1) *BMJ Digital Health & AI* e000092. https://bmjdigitalhealth.bmj.com/content/1/1/e000092; Joshua Biro and others, "Accuracy and Safety of AI-Enabled Scribe Technology: Instrument Validation Study" (2025) 27 *J Med Internet Res* e64993 https://doi.org/10.2196/64993; Erin Palm and others, "Assessing the Quality of AI-Generated Clinical Notes: Validated Evaluation of a Large Language Model Ambient Scribe" (2025) 8 *Front Artif Intell* 1691499 https://doi.org/10.3389/frai.2025.1691499 (all accessed 12 May 2026).

[12] NHS England, *Guidance on the Use of AI-Enabled Ambient Scribing Products in Health and Care Settings* (Version 2, PRN01734_i, NHS England, first published 27 April 2025, last updated 24 April 2026) https://www.england.nhs.uk/long-read/guidance-on-the-use-of-ai-enabled-ambient-scribing-products-in-health-and-care-settings/, accessed 12 May 2026 (NHS England, *AI-Enabled Ambient Scribing Guidance*).

[13] Royal Australian College of GPs, *AI Scribes Fact Sheet*, updated 27 October 2025 https://www.racgp.org.au/running-a-practice/technology/artificial-intelligence-ai/artificial-intelligence-ai-scribes.

[14] NHS England, *AI-Enabled Ambient Scribing Guidance.*

3.2 Privacy

It is also important that patient data used by doctors and third parties in the context of AI scribes meets patients' reasonable expectations of confidentiality and complies with applicable legislation, such as the Australian Privacy Act and the APPs.

Our research suggests that appropriate, specific, and granular consent for the collection of sensitive health information is generally not being sought by clinicians in line with APP3 and guidance from the privacy regulator, the OAIC.[15] In certain Australian jurisdictions, the recording of a private conversation without the consent of the parties involved may even constitute a criminal offence.[16]

There is also no clarity regarding the right to refuse this type of processing. Recently, an Australian who refused the use of an AI notetaker was told that the medical practice could not provide services as a result.[17] Concerns have also been raised by experts about whether consent can be legitimate and genuine given the power imbalance between doctors and patients. Elizabeth Deveny, CEO of the Consumers Health Forum, stated in an article in the Medical Journal of Australia that, given these power imbalances, a rushed 'This okay with you?' at the start of a consultation may not constitute meaningful consent.[18]

There are also legal requirements for certain information to be included in privacy policies under APP1.4, including how personal information is collected and held (in this case by AI scribes or audio recording). The OAIC's guidance on the deployment of commercially available AI products requires privacy policies be updated to include their use of AI. Finally, APP5 requires notification of the collection of personal information, including the circumstances and purposes of the collection.

Our research found that medical practices in Australia using AI tools generally appear to not have updated their privacy policies to reflect this processing.

Our Research Findings on Privacy Compliance

We reviewed the privacy policies of a number of Australian medical providers who feature on the websites of these AI tools as customers, or are otherwise identified as using AI scribes. **Of the 60 privacy policies we reviewed, only 4 explained the use of AI scribes.**

We also reviewed the data protection, and privacy information and policies of 7 major AI scribe providers in the Australian market. We identified a number of issues that cut across the market:

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[15] OAIC, "Consent to the handling of personal information guidance", accessed 18 May 2026 https://www.oaic.gov.au/privacy/your-privacy-rights/your-personal-information/consent-to-the-handling-of-personal-information.

[16] RACGP, *AI Scribes.*

[17] Tom Williams, "Kobi Refused a Doctor's AI. She Was Told to Go Elsewhere" (*Information Age*, 12 August 2025) https://ia.acs.org.au/article/2025/kobi-refused-a-doctors-ai-she-was-told-to-go-elsewhere.html, accessed 12 May 2026.

[18] Elizabeth Deveny, "AI scribes in general practice: support, silence and the shape of care", InSight+ (MJA) (21 July 2025) https://insightplus.mja.com.au/2025/28/ai-scribes-in-general-practice-support-silence-and-the-shape-of-care, accessed 13 May 2026.

- Many AI scribe providers set default retention periods and allow users to adjust them based on their preferences. However, these retention periods do not appear to apply to audio recordings: most, if not all, AI scribe providers explicitly claim to discard audio as soon as it is transcribed. While framed as data minimisation, this can limit accuracy checks if errors occur and audio cannot be reviewed.

- Many AI scribes state they store data locally, in Australia. However, most also mention the use of third-party vendors, without making clear where these vendors are based. Given the supply chain of this kind of technology – which most likely relies on transcriptions being sent to model providers via API calls – it is possible that data may be sent overseas triggering APP requirements and additional risk.

- Some AI scribes use patient data beyond transcription (e.g., for analytics, product development, model training), often without providing clear detail. This may require separate, specific patient consent under current laws.

- It is unclear whether AI scribe providers de-identify data before using it to train AI models. If companies are doing this, it raises significant concerns both in terms of re-identification as well as whether removing a name or identifiers from a patient and inserting that data into a system is ethical or appropriate, given the sensitive personal nature of medical experiences and conditions. De-identified data may still be able to be re-identified (as opposed to anonymised data).

3.3 Other Risks

AI scribes may introduce new functions unintentionally or through user-provided instructions, generating outputs that go beyond the product's intended purpose, even without deliberate intent by the user.[19] Another underlying risk related to AI scribes is that, as adoption increases, GPs may develop over-reliance on these tools, resulting in reduced attention to critical clinical details or failure to adequately review AI-generated outputs. The act of manually drafting clinical documentation may itself serve a cognitive function, supporting GPs in developing a clinical formulation; the consequences of displacing this process remain insufficiently understood.[20]

A significant underlying risk is the potential 'chilling effect' of patients no longer feeling able to freely express themselves when being recorded. While further research is needed, patients may self-censor and choose not to disclose certain information when they are being recorded – whether about domestic violence, child protection, sexual health, stigmatised conditions, drug and alcohol consumption, or other sensitive issues. Experts have noted that healthcare is deeply human; consultations are more than mere transactions or information

[19] NHS England, *AI-Enabled Ambient Scribing Guidance.*
[20] RACGP, *AI Scribes.*

exchange.[21] The introduction of AI into consultations changes not only what gets recorded, but what gets said.[22]

4 Using AI Scribes Responsibly as GPs and Medical Practices

Medical Practices:

Product assessment/procurement

- ❑ **Consider your need for an AI scribe**: Develop a clear picture of your functional requirements prior to market engagement. Determine whether an AI scribe is needed at all, and if so, whether your needs can be met by an off-the-shelf solution, or require a degree of tailored development or configuration– this may have direct implications for contracting, liability, and ongoing maintenance obligations.[23]

- ❑ **Consider the training data used:** Check whether the AI scribe product has been specifically tested and validated for use with the patient population your practice serves, including with respect to the diversity of training data used to develop the underlying models. The OAIC notes that an AI system trained on non-Australian datasets may not comply with APP10 accuracy obligations when applied to an Australian clinical setting. You should ask vendors to provide documentation evidencing the diversity, provenance, and relevance of their training data.[24]

- ❑ **Interoperability**: Before engaging any supplier, including for a pilot, undertake a structured assessment of interoperability requirements between the AI scribe and your critical systems, particularly your EHR or practice management system.[25]

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[21] Elizabeth Deveny, “AI scribes in general practice: support, silence and the shape of care”, InSight+ (MJA) (21 July 2025) https://insightplus.mja.com.au/2025/28/ai-scribes-in-general-practice-support-silence-and-the-shape-of-care, accessed 13 May 2026.
[22] Elizabeth Deveny, “AI scribes in general practice: support, silence and the shape of care”, InSight+ (MJA) (21 July 2025) https://insightplus.mja.com.au/2025/28/ai-scribes-in-general-practice-support-silence-and-the-shape-of-care, accessed 13 May 2026.
[23] NHS England, *AI-Enabled Ambient Scribing Guidance.*
[24] Office of the Australian Information Commissioner, *Guidance on Privacy and the Use of Commercially Available AI Products* (OAIC, first published 21 October 2024, updated 17 January 2025). https://www.oaic.gov.au/privacy/privacy-guidance-for-organisations-and-government-agencies/guidance-on-privacy-and-the-use-of-commercially-available-ai-products, accessed 12 May 2026 (OAIC, *Commercially Available AI Products Guidance*).
[25] NHS England, *AI-Enabled Ambient Scribing Guidance.*

- ❑ **EHR integration**: When evaluating vendors, assess their experience with EHR or practice management system integration (as relevant), and their product performance, system availability, response time, and scalability assurances.[26] Check whether the product can both send data to, and receive data from, your EHR. The system should be able to automatically align with your organisation's clinical documentation standards by writing notes directly into the correct patient record. Systems lacking this integration increase the risk of notes being recorded in the wrong patient record or errors being introduced with a manual copy-paste step.[27]

- ❑ **Consider Therapeutic Goods Act (TGA) 1989 (Cth) requirements**: Assess whether the AI scribe may qualify as a medical device under TGA requirements. This determination should be made independently of any representations by the vendor. Products using generative AI beyond transcription and administrative support likely fall under TGA requirements. For example, a product which analyses clinical conversations or notes for the purpose of determining, suggesting, or supporting diagnoses or treatment plans.

- ❑ **Undertake vendor risk management and due diligence**: Before piloting or procuring an AI scribe, conduct structured due diligence to ensure it can meet the relevant requirements. This should cover, at minimum: data localisation and the jurisdictions in which any third-party sub-processors operate (Privacy Act 1988 (Cth), APP8); data retention periods across all data types (audio, transcripts, and generated notes); whether patient data is used for secondary purposes such as model training, and the ability to opt out of such processing (APP6); and the existence of a compliant data processing agreement. This should also include a review of any standard contractual documentation, such as Terms of Use, to determine whether patients' information may be used for secondary purposes by the vendor or third parties,[28] as well as a review of the same documents to evaluate whether they grant the developer (or any third-party) access to personal information entered into or generated by the system, in which specific instances this may happen, and whether this access can be disabled.[29] Please see Annex IV for vendor due diligence checklist.

- ❑ **Privacy Impact Assessments (PIA)**: While not strictly required under Australian law (other than for public agencies where there is a high risk to privacy), conducting a PIA prior to deploying an AI scribe is strongly recommended as a matter of best practice. A PIA should assess the lawful basis for collecting and processing patient health information, the risks arising from third-party involvement, and the adequacy of the technical and organisational measures in place to protect that information and the rights of data subjects.

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[26] NHS England, *AI-Enabled Ambient Scribing Guidance.*
[27] NHS England, *AI-Enabled Ambient Scribing Guidance.*
[28] RACGP, *AI Scribes.*
[29] OAIC, *Commercially Available AI Products Guidance.*

Compliance readiness

- ❑ **Engage legal counsel**: Do this prior to procurement, in order to assess liability exposure arising from the use of AI scribes, since the GP, practice or employing organisation may ultimately bear residual liability where the product supplier does not fully assume responsibility or lacks adequate coverage.[30]

- ❑ **Supplier contracts**: Ensure supplier contracts include clear and comprehensive provisions on roles, responsibilities, and liability – particularly where the practice customises product outputs (e.g., developing tailored templates), as this can affect the allocation of responsibility for errors arising from those customisations.[31] Ensure that contractual documentation includes applicable Service Level Agreements (SLAs), which should specify minimum system uptime, incident response times, software update commitments, and ongoing training obligations for both clinical and administrative staff.[32]

- ❑ **Get your privacy compliance in order**: Ensure your practice's privacy framework is updated to reflect the use of AI scribes before deployment. This includes: confirming that the collection of patient health information via an AI scribe is either consented to or otherwise permitted under the conditions applicable to sensitive information (APP3); updating your privacy policy to specifically disclose AI scribe use, ensuring this specifically identifies that patient health information is collected via an AI scribe system, that the AI system developer is identified as a recipient of that information, and that any secondary purposes for which the information may be used or disclosed (including any model training or product improvement purposes)[33] are laid out (APP1); notifying patients of the collection at or before the point of recording (APP5); ensuring patient data is not used or disclosed by the vendor for purposes beyond those for which it was collected, such as model training or product analytics (APP6). This step should also include implementing all the necessary policies and processes in relation to, for example, detecting and managing security incidents, and receiving and responding to requests from individuals in relation to your use of their personal information. Please see annexes which provide relevant templates to be tailored as appropriate.

- ❑ **Consider AI watermarking**: AI-generated clinical notes and transcriptions should not be treated or recorded as established facts[34] where they have not been reviewed and verified by a clinician. Where such records are the product of a probabilistic AI output, they should be clearly marked as created using AI. Even once verified by a clinician, notes should be marked as created with the help of AI, including by identifying the data source and the AI system used to generate them.

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[30] NHS England, *AI-Enabled Ambient Scribing Guidance.*
[31] NHS England, *AI-Enabled Ambient Scribing Guidance.*
[32] NHS England, *AI-Enabled Ambient Scribing Guidance.*
[33] OAIC, *Commercially Available AI Products Guidance*.
[34] OAIC, *Commercially Available AI Products Guidance*.

Organisational readiness

- ❑ **AI scribes whitelist**: Following completion of the vendor due diligence process, maintain a practice-level register of AI scribe products that have been formally assessed and approved for use. Staff should only use tools from this list, and the register should be reviewed periodically and updated to reflect any material changes to vendors' products, terms, or data practices.

- ❑ **Use case inventory with best practices**: Maintain a practice-level register of approved AI scribe use cases, documenting the clinical contexts in which the technology has been assessed as appropriate for use, and the specific conditions and safeguards that apply. The register should be reviewed periodically as the technology and your practice's experience with it evolves.

- ❑ **Training and awareness**: Users receive structured training on how to use AI scribes prior to deployment. This should cover appropriate use of the tool in a clinical setting, including how to appropriately obtain consent, how to answer basic questions like where personal data is stored, and how long it is retained, as well as consultation behaviour, dictation technique, and how microphone proximity and positioning can affect transcription accuracy. Whenever significant updates to the AI scribe are made, training should be refreshed, with support from the AI scribe vendor, if relevant. Training should reinforce clinicians' ongoing professional responsibility to review and revise all AI-generated outputs before they are saved to the patient record, and should include guidance on how to obtain patient consent to use an AI scribe.[35]

- ❑ **Establish and maintain a clear internal policy**: Define the permitted and prohibited uses of AI scribes within the practice, including explicit prohibitions on using AI scribe outputs as a substitute for clinical judgment or as the basis for decisions without human verification. This policy should be communicated to all clinical and administrative staff and reviewed periodically.[36]

Quality and performance monitoring

- ❑ **Conduct independent monitoring of AI scribe performance**: This should be separate from any metrics reported by the manufacturer and include error reporting processes, service usage monitoring, and periodic comparison of reported versus observed performance metrics. This monitoring should include patient complaints, consent issues, privacy incidents, assessment of clinician reliance levels, and workflow impact. It should confirm that the product works adequately for the patient population your practice serves, including those with non-standard accents, dialects, or speech

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[35] NHS England, *AI-Enabled Ambient Scribing Guidance*, RACGP, *AI Scribes.*
[36] OAIC, *Commercially Available AI Products Guidance*; NHS England, *AI-Enabled Ambient Scribing Guidance.*

disorders.[37] You should include mechanisms such as periodic audits of notes and spot checks, as well as surveying patients and doctors on the use of AI scribes.

GPs and Clinicians:

- ❑ **Carry out human oversight of AI-generated content:** Always review the full transcript and documents against your own recollection of the consultation. Pay special attention to medication names, dosages, and clinical terms. Edit AI-generated notes to use patient-friendly, person-centred language before saving. Monitor whether the tool performs differently for patients with diverse backgrounds or accents. Report recurring errors and patterns of bias to your practice manager and vendor. Supplement AI notes with your own observations where you notice lower accuracy. Ensure the position and proximity of the recording device is appropriate for the consultation setting, including accounting for any changes to usual ambient noise levels.

- ❑ **Clearly communicate data use to patients:** Ensure your practice has a clear privacy notice that specifically covers AI transcription and display signage in the waiting room. See Annex II for an example waiting room notice template. Include AI scribe use in your collection notification to patients under APP5. Patients should be provided with explanations in simple terms of the technology used, how it works, what its use entails (e.g., in terms of third parties such as the AI scribe provider having access to their health data), as well as the risks in relation to its use.

- ❑ **Obtain informed consent:** Explain to patients in plain language what the AI scribe does, how their data is used, and who can access it. Obtain explicit consent before activating recording. Offer a genuine alternative (on-device transcription or manual notes) for patients who decline. Ensure that consent is granular – i.e., the consent you request for using the AI scribe needs to be separate from any consent the patient needs to give you to allow you to process their personal information. Remember that, even if using an AI scribe would be more convenient for you, you cannot refuse to provide treatment if a patient does not want you to use it. Emphasise the ability to say ‘no’ or to request at any time that the recording be turned off, including where a patient may want to discuss something sensitive. See Annex I for an example script for obtaining patient consent.

- ❑ **Ensure reasonable data retention:** Review the default retention periods provided by your vendor and ensure they are in line with your corporate policies and/or are not excessive in relation to your need to retain them for the future. Consider also that once the information has been exported to your EHR and is safely backed up in another location, there may be no further need to retain it on your AI scribe storage.

[37] NHS England, *AI-Enabled Ambient Scribing Guidance.*

Ask your AI scribe provider to explain if and how you can review audio recordings of consultations to ensure the accuracy of transcriptions and notes based on the recording.

- ❑ **Exercise data minimisation:** Review your vendor's data minimisation practices and ensure data is not used for secondary purposes. Ask your vendor, if necessary, whether they process your patient health information for AI training or any other product improvement purposes. If so, request to opt out of any such processing.

- ❑ **Maintain data processing within Australia if possible, including in relation to third-party sub-processors:** Before procuring an AI scribe, confirm where data is stored and processed. Require Australia-based data residency or equivalent protections in your vendor contract. If you work for a hospital or a clinic, request that the relevant person in your practice (e.g., general counsel, head of procurement) conducts a privacy impact assessment.

- ❑ **Be aware of potential loss of clinical judgement and deskilling:** Continue to exercise your own clinical judgement when reviewing AI outputs. Periodically write consultation notes manually to maintain your documentation skills. Use AI tools as assistants, not as replacements for your professional analysis. AI scribes may induce automation bias and subtly influence your clinical reasoning in ways that are not immediately apparent. Use the AI scribe as a documentation tool, not a clinical decision support system, unless it has been specifically validated and approved for that function.[38]

- ❑ **Provide patients with the opportunity to correct or delete AI-processed data:** Establish a clear process for patients to request access to, and correction of, AI-generated records. Understand your vendor's data retention and deletion policies. Ensure your practice's data governance policy covers AI-generated content.

- ❑ **Check the implications of medical devices regulation:** Check whether your AI scribe vendor has any clinical safety certifications or compliance standards. Advocate through your professional body for clearer regulation of AI tools in Australian healthcare. Do not use AI scribes for diagnostic functions for which they are not designed.

- ❑ **Avoid excessive ambient data collection:** Only activate the AI scribe during the clinical consultation, not before or after. Use the tool in a private room to avoid capturing third-party conversations.

[38] NHS England, *AI-Enabled Ambient Scribing Guidance.*

- ❑ **Use easy-to-understand language for records:** Edit AI-generated notes to use patient-friendly, person-centred language before saving. Use prompts or templates that instruct the AI to write in plain language. Remember that patients have a right to access their records and should be able to understand them.

5 FAQs for GPs and Medical Practices

How do I determine if it is appropriate to use an AI scribe?

It is best to treat AI scribes as tools that augment your work rather than tools that can fully replace your own independent thinking and judgment. This means that you should only use AI scribes if you are prepared to check the outputs generated by the tool; you should not be using AI scribes without this verification step. This is in addition to other steps you should be taking when using these tools, including ensuring that they meet security and compliance requirements by undertaking vendor due diligence, as well as obtaining consent and being transparent with patients about how you use AI scribes and the handling of their information. You should always give patients a real chance to say 'no'.

Are there any privacy preserving alternatives?

While issues related to reliability and accuracy are inherent to the technology, and can therefore only be mitigated by human review, the privacy risks related to the use of AI scribes can be substantially mitigated by relying on transcription and note generation software that works locally on your device.

This is because the models used by these tools are stored and run locally on the user's device, without needing to transfer any data elsewhere. Moreover, these systems retain the audio recording of the consultation/dictation and may offer functions like linking time-stamps from the recording to the written text – which can make the human review process more accurate as GPs can listen to the original recording to correct any inaccurate transcription.

Certain tools are presented as fully on-device AI scribe solutions, such as PrivateScribe[39] and Pallin/TranscribePad.[40] Repositories such as GitHub also share other local AI scribe solutions, such as Phlox[41] and Offline Medical Scribe.[42]

These on-device solutions can provide transcription and note-generation functionalities that are functionally equivalent to those of other AI scribes, while guaranteeing that you are in full control of the data of your patients.

The privacy-preserving nature of local deployment has also been recognised in the OAIC guidance on AI systems.[43]

How do I obtain consent from patients? What information do I need to give them?

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[39] https://www.privatescribe.ai/
[40] https://www.transcribepad.com/
[41] https://phlox.bloodworks.io/
[42] https://github.com/harishkotra/offline-medical-scribe
[43] OAIC, *Commercially Available AI Products Guidance.*

Based on the OAIC guidance, you should consider the following when asking for your patients' consent to the processing of their health information:

- **Prior**: You need to collect consent before commencing the processing of your patients' personal information.
- **Informed**: Before asking for consent, you need to provide your patients with clear information on how you intend to handle their information.
- **Voluntary**: Your patients shouldn't feel pressured into providing consent.
- **Express**: Consent needs to be given openly and obviously, either verbally or in writing, for example by signature.
- **Unbundled**: You will likely have to ask for your patients' consent for different processing and/or purposes (e.g., providing treatment, conducting research, disclosing information to an AI scribe vendor). You should always allow patients to agree separately to these different types of processing, to ensure they have a genuine choice.
- **Withdrawal**: Patients can withdraw their consent at any time. You should provide an easy and accessible process for them to follow, and ensure you do not process their information for that specific purpose/process. Emphasise the ability to say 'no' or to request at any time that the recording be turned off, including where a patient may want to discuss something sensitive.

When you are dealing with underage patients, or patients who do not have capacity, you should seek their guardian's or 'responsible person's' consent.

How do I select an AI scribe tool?

If you belong to a practice, the selection process should be carried out at the practice level. As a clinician, you should only use those tools that have been cleared by your practice.

If you are an independent practitioner, when procuring a tool you should consider the following:

- Whether the tool is adequate for your **implementation requirements**, such as the possibility of directly integrating with your EHR system, and whether the vendor can assure adequate product performance, system availability, and response times.
- Whether the vendor can ensure an adequate level of **accuracy**. You can do this by reviewing any information provided by the vendor in relation to the development and testing of the AI scribe, checking in particular whether the product has been tested and validated for use with the patient population you serve.
- Assess whether the AI scribe may qualify as a **medical device** under TGA requirements (e.g. because it provides medical coding suggestions or treatment recommendations). This determination should be made independently of any representations by the vendor.

- Assess whether the **privacy safeguards** implemented by the vendor comply with the APPs – e.g., where the vendor stores the data and the jurisdictions in which any third-party sub-processors operate; the duration of data retention periods and whether they can be customised; whether patient data is used for secondary purposes such as model training or product development, and whether you can opt out; and whether the vendor provides a compliant data processing agreement.
- Assess whether the contracts and documentation provided by the vendor provide acceptable **liability** exposure, for example in the case of issues arising from inaccurate AI scribe outputs.

Are AI scribes in healthcare regulated in Australia?

Although there is currently **no dedicated regulatory framework** for AI scribes in Australia, several existing instruments are still relevant to their use. These include:

- **The Australian Privacy Act 1988 (Cth)**. This governs the collection, use, and disclosure of personal information, including the patient health information processed by medical practices and AI scribe vendors. The Australian Privacy Act contains the 13 Australian Privacy Principles (APPs).
- **The Therapeutic Goods Act 1989 (Cth)**. The TGA has commenced a review of digital scribes in Australia. has confirmed these tools meet the definition of a medical device if they are intended to perform any of the following functions:
  - Diagnose, monitor, predict, provide a prognosis, treat, alleviate or compensate for a disease, injury or disability;
  - Investigate anatomy or physiological processes;
  - Control or support conception;
  - Examine specimens derived from the human body for a medical purpose.

Therefore, tools that incorporate diagnostic support functionalities (e.g., medical coding suggestions, treatment recommendations) would likely still meet the threshold for classification as a medical device under current requirements.

What are the risks?

Generally, the main risks related to the use of AI scribes are the following:

- **Accuracy and bias**: AI scribes may transcribe incorrectly – omitting or misinterpreting key information, as well as fabricating details never spoken. This may lead to lower performance for patients with non-standard accents, dialects, or speech impairments, disproportionately affecting certain patient groups.
- **Privacy**: Whenever you engage an AI scribe provider, patient health information is transferred to a third-party vendor which may, in turn, rely on subcontractors in a foreign jurisdiction that process the information (either by receiving it or by having access to it). Vendors may also use your patients' information for secondary purposes, including model training or new product development. Inadequate privacy notices and consent practices may create significant compliance risks under the APPs.

- **Autonomy**: Individuals may feel less comfortable or surveilled and, as a result, may choose not to disclose certain important information, including about domestic violence, child protection, sexual health, stigmatised conditions, drug and alcohol consumption, or other sensitive issues.

Who is responsible for ensuring accuracy?

Although there may be instances in which you make claims against a vendor for inaccuracies in an AI scribe's output, based on the current terms provided by vendors and the fact that AI scribes are not currently regulated as therapeutic goods, in most instances **GPs retain full responsibility for any inaccuracies in their consultation notes**.

Ultimately, as the treating clinician, these are *your* notes, and *you* remain professionally responsible and legally liable for the accuracy and completeness of all clinical documentation, regardless of whether its generation was aided by an AI scribe.

You should therefore always treat the output of these systems as preliminary. In practice, this means you should:

- Read every AI-generated note in full before saving it to the patient record;
- Ensure that the AI scribe did not miss out any relevant detail, misinterpret a word or sentence, or add information out of context;
- Pay particular attention to medication names, dosages, diagnoses, and clinical terms;
- Supplement AI notes with your own clinical observations, particularly when you notice low transcription accuracy;
- Report recurring errors and patterns to your practice manager and vendor; and
- Never use an AI scribe output as a substitute for clinical judgement or as the basis for a decision.

Is there any other guidance that I can rely on?

Yes. The following authoritative sources provide further guidance relevant to the use of AI scribes in clinical settings:

- **RACGP – Artificial intelligence (AI) scribes**
  https://www.racgp.org.au/running-a-practice/technology/artificial-intelligence-ai/artificial-intelligence-ai-scribes

- The **ACSQHC** has published an **AI Clinical Use Guide: Guidance for Clinicians** https://www.safetyandquality.gov.au/sites/default/files/resources/attachments/ai-clinical-use-guide.pdf to help clinicians understand what to consider before, during and after using AI in clinical care, alongside a separate **AI Safety Scenario: Ambient scribes**
  https://www.safetyandquality.gov.au/sites/default/files/resources/additional/ai-safety-scenario-ambient-scribe.pdf

- **NHS England – Guidance on the use of AI-enabled ambient scribing products in health and care settings**
  https://www.england.nhs.uk/long-read/guidance-on-the-use-of-ai-enabled-ambient-scribing-products-in-health-and-care-settings/

- **OAIC – Guidance on privacy and the use of commercially available AI products** https://www.oaic.gov.au/privacy/privacy-guidance-for-organisations-and-government-agencies/guidance-on-privacy-and-the-use-of-commercially-available-ai-products
- The **Therapeutic Goods Administration** has published **Digital scribes: When digital scribes are regulated as medical devices** for queries regarding the classification of AI tools as medical devices under the TGA 1989 (Cth) https://www.tga.gov.au/products/medical-devices/software-and-artificial-intelligence-ai/overview/types-software-based-medical-devices/digital-scribes
- **The Australian Signals Directorate – Guidance on how to use AI systems securely** https://www.cyber.gov.au/business-government/secure-design/artificial-intelligence/engaging-with-artificial-intelligence /

Annex I – Patient consent script

[Please note, this consent should be tailored to the specifics of the processing and AI scribe tool you are using. It is your responsibility to ensure it meets the requirements of consent under Australian privacy law and OAIC guidance. This template provides a starting point to work from and should not replace legal advice.]

Before we begin, I would like to ask you if you are ok with me using [insert name of AI scribe tool], which is an AI scribe to help me document our consultation. The AI scribe will record our conversation and automatically generate a transcript. I will later use it to automatically generate clinical notes and other clinical documentation, such as referral letters or consultation reports, which I will review and share/save to your medical record accordingly.

The recording and notes will be processed by a third-party software provider, [insert vendor name], whose servers are located in [insert jurisdiction]. Your health information may also be accessible to their subcontractors, who are located in [insert jurisdiction]. You can learn more on the functioning of AI scribes and the information they process by looking into [insert reference to practice privacy policy/other explainer on AI scribes].

[Your health information will be used to generate your consultation notes. It will not be used for any other purpose, such as AI model training, without your separate and specific consent.]

You have the right to decline the use of this tool. If you prefer, I can [use an on-device speech-to-text software that will produce a transcription of our conversation, with your data never leaving my device or] take notes manually. Declining will not affect the care you receive in any way. You can also ask me at any time to turn it off, including if you want to discuss anything particularly sensitive.

Do you consent to the use of this AI scribe for the consultation?

[Consent/refusal should then be recorded, e.g., in a spreadsheet or on the system.]

Annex II – Waiting room notice

[Please note, this notice should be tailored to the specifics of the processing and AI scribe tool you are using. It is your responsibility to ensure that it meets the requirements of APP1 and APP5. This template provides a starting point to work from and should not replace legal advice.]

Notice: This clinic is using [insert name of tool], an AI scribe/ AI notetaker

To support clinical documentation, your doctor may use an AI scribe tool during your consultation. The doctor will ask for your consent – you do NOT need to say yes and can refuse consent at any time.

What is an AI scribe?

An AI scribe is a tool that aids clinicians by recording and transcribing patient consultations, and automating the drafting of clinical notes and other documentation.

What this means for you

If you consent to the use of the AI scribe, your health information will be shared with [name of AI scribe vendor], which provides [name of AI scribe]. This is the tool we use at [name of practice] to assist with clinical documentation. Your information will also be shared with [name of AI scribe vendor]'s subcontractors, [name of subcontractors].

The AI scribe does not replace your doctor's clinical judgement and your doctor is responsible for reviewing all medical notes on your file.

Your privacy

The tool has been assessed by our practice to meet security, confidentiality, and privacy requirements.

You can learn more about how we handle your personal information by requesting a copy of our privacy policy at the information desk or consulting it at [insert link].

Storage and disclosure

Some subcontractors engaged by [name of AI scribe vendor] are located outside Australia. Where this is the case, [name of AI scribe vendor] is contractually required to ensure that equivalent privacy protections apply.

Your choice

You can choose not to have this tool used during your consultation.

Please let your doctor know if you would prefer this. You can also ask your doctor to stop using the AI scribe at any time during your consultation.

Access and questions

A W O

You can request access to your personal information including your health record and ask to correct your personal information at any time.

To do this, or in case you have questions or concerns, please speak to our staff or contact: [practice name / Privacy Officer contact details].

Annex III – Template privacy policy wording on the use of AI scribes

[To be amended and appended to the text of the practice's main privacy policy, which should include information such as contact details, the right to complain, and to access and correct personal data. Please note, this wording should be tailored to the specifics of the processing and AI scribe tool you are using. It is your responsibility to ensure it meets the requirements of APP1 and fits with your broader policy. This template is a starting point to work from and should not replace legal advice.]

Use of AI scribes [and documentation tools]

[Name of practice] uses [name of AI scribe], provided by [name of vendor], as a tool to facilitate consultation transcriptions and note generation. You can learn more about [name of AI scribe] at their website [insert relevant link to vendor website].

What personal information is collected, and how it is processed by the AI scribe?

When an AI scribe is used during a consultation, our GP's collect health information disclosed during the consultation. This may include symptoms, diagnoses, treatment suggestions, or medication names. As part of the process, the AI scribe processes the audio recording of your consultation and transforms it into a transcript. Your doctor can then prompt the system to produce clinical notes or other documentation based on the transcription. [The doctor can upload additional documentation and refer to previous consultations to produce further notes, or interrogate the system about your medical history.]

Secondary use

Your health information [will/will not] be used by [name of vendor] for secondary purposes, including AI model training, product improvement or development. [We will ask for your separate consent for this processing purpose.]

Access, storage and overseas disclosure

As part of this process, your health information is disclosed to [name of vendor], and its subcontractors.

Data is stored by [name of vendor] on servers located in [jurisdiction]. The subcontractors operating on its behalf may access your information from [insert jurisdiction]. Where personal information is disclosed overseas, we have taken reasonable contractual steps to ensure protections equivalent to those required under Australian law apply to overseas processing.

Retention

Audio recordings of consultations are [deleted immediately following transcription/retained for X amount of time]. Transcripts and clinical notes are retained for [insert specific retention period or reference to specific applicable legal and professional requirements].

Annex IV – Vendor due diligence checklist

[Please amend as appropriate for your organisation. You may have a policy that governs this kind of vendor diligence. The user-friendly checklist below is intended as a guide. It does not replace comprehensive legal advice or internal policies.]

1. Accuracy and performance

- ❑ Can the vendor provide documentation on the accuracy of their product, including error rates and other metrics, training data diversity, provenance, and relevance?
- ❑ Has the product been tested and validated for use with the patient population the practice serves, including patients with non-standard accents, dialects, or speech impairments?

2. Medical device classification

- ❑ Does the product include diagnostic support functionalities (e.g., medical coding suggestions, treatment recommendations)?
- ❑ What does the vendor documentation state in relation to the medical device status of its product?

3. Contractual safeguards

- ❑ Does the vendor provide a data processing agreement?
- ❑ Are liability provisions clearly defined and in line with medical practice regulations?
- ❑ Do contracts include enforceable service level agreements (SLAs) covering uptime, incident response, and update obligations?

4. Data localisation and sub-processors

- ❑ In which country does the AI scribe provider store consultation data?
- ❑ From which country does the AI scribe provider access consultation data?
- ❑ Does the vendor use subcontractors to provide the AI scribe service? If so:
    - o From which country/ies do the subcontractors access consultation data?
- ❑ Where data is processed outside Australia, are adequate data safeguards implemented by the AI scribe provider and its subcontractors?

5. Data retention

- ❑ What are the default retention periods for audio recordings, transcripts, and generated notes?
- ❑ Can audio recordings be accessed by the clinician after transcription to verify accuracy?
- ❑ Can retention periods be customised?

6. Secondary use of patient data

- ❑ Does the vendor use patient health information for any secondary purpose, including AI model training, or product improvement or development?
- ❑ Can the practice opt out of any such secondary processing?